



Human Rights, Disability, and Mindfulness

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Abstract

Objective The Convention on the Rights of Persons with Disabilities (CRPD) has identified and articulated the rights of people with disabilities, mandating how these rights should be upheld and protected by countries. The objective of this article is to explore how these rights can be translated into action for people with intellectual and developmental disabilities (IDD).

Method We conceptualize what human rights mean in the context of people with IDD, examine models of disability that have been utilized to provide services, and explore how mindfulness research and practices could be better aligned with the CRPD.

Results We present five core principles of interdependent human rights of people with IDD and highlight five key challenges that hinder the realization of their fundamental rights and freedoms, which are intrinsic to the dignity of all human beings and are an essential component of a just and equitable society. We delve into the key Articles of the CRPD that underpin these rights and freedoms. Various models of services for people with IDD have been developed and implemented. We underscore the strengths and limitations of the medical, social, and biopsychosocial models, and provide an overview of the emergent human rights model that is based on the CRPD. Mindfulness is discussed in the context of other interventions as an adjunctive treatment and a potential intervention that could address some concerns expressed in the CRPD.

Conclusions Human rights, disability, and mindfulness are inextricably linked and could be strengthened to advance equity and full inclusion of people with IDD in our society. Impairment cannot be used as a basis for denying or restricting their rights. We need to promote the understanding that disability is an essential part of human diversity, and people with disabilities contribute unique and crucial perspectives in a just and enlightened society.

Keywords CRPD · Human rights · Disabilities · People with IDD · Medical model · Social model · Biopsychosocial model · Human rights model · Mindfulness

People with IDD have been on a long journey through the gray areas of life, often experiencing exclusion, marginalization, institutionalization, socially devaluation, and discrimination by society. They have endured low quality of life over several centuries (Cummins, 2016). Various international bodies have proposed principles and practices that should have changed the way society has treated people with IDD. Although change has been palpable in recent years, it has been very slow, indicating that the rights of people

with IDD as citizens have yet to be fully realized. We discuss these issues within the context of rights that should be afforded to all people, including those with IDD, the nature of disability, and how mindfulness-based interventions may offer them some hope of controlling the trajectory of their own lives.

We acknowledge the significant role language plays in shaping the self-perception of disabled people and influencing how disability is perceived by others. In this article, we use a combination of person-first and identity-first terminology in accordance with the recommendations of the American Psychological Association regarding disability-related language.

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Human Rights

Human rights underpin the fundamental rights and freedoms of every individual regardless of their race, ethnicity, sexual orientation, gender identity, nationality, religion, or any other distinguishing characteristic, including all forms and aspects of disabilities. These rights are intrinsic to the dignity of every human being and are an essential component of a just and equitable society. The concept of human rights originated in ancient times, with Code of Hammurabi from Babylonian legal document (1755–1750 BC) being one of earliest. Others followed, including the Magna Carta (1215), the English Bill of Rights (1689), the American Declaration of Independence (1776), and the French Declaration of the Rights of Man and of the Citizen (1789), which codified individual liberty and limitations on government power. Much later, the Universal Declaration of Human Rights was adopted (1948) by the United Nations following the unthinkable horrors of World War II and the Holocaust (see Parmenter, 2023 for a brief history of the concept of human rights).

“Human rights” is a fairly amorphous term that has been conceptualized differently across countries and societies with different cultural and political orientations. However, there are five core principles that overlap across nations: (1) Human rights are universal and applicable to everyone regardless of personal characteristics; (2) These rights, in particular human dignity, are inalienable and not contingent on a person’s behavior or actions; (3) All such rights are indivisible, meaning that a person’s civil and political rights (e.g., freedom of speech and right to a fair trial) are inextricably linked with economic, social, and cultural rights (e.g., right to healthcare and education); (4) These rights are interdependent in the sense that the fulfilment of a specific right is dependent on the fulfilment of other rights; and (5) In truly civilized societies, all human rights are guaranteed and non-discriminatory concerning a person’s characteristics (e.g., race, ethnicity, faith, sexual orientation, and all other distinguishing characteristics).

In theory, human rights declarations codify universal entitlements for all people, regardless of their abilities, disabilities, or characteristics. In practice, such entitlements are violated across all societies and countries, given political and economic realities, wars, effects of climate change, and a range of systemic discriminatory practices. For example, people with intellectual disabilities (ID), developmental disabilities (DD), and intellectual and developmental disabilities (IDD) have been subjected to exclusion, neglect, and institutionalization, resulting in compromised quality of life (Cummins, 2016). Although significant progress has been evidenced in some countries, many challenges remain, including: (1) stigma and discrimination (e.g.,

discrimination in educational practices, hiring and work conditions in employment, and access to public services); (2) public education (e.g., inclusive education is still an aspiration in many countries, with limited access to quality education); (3) access to employment (e.g., finding suitable employment is difficult because employers discriminate and underestimate their abilities and capabilities in fulfilling work requirements); (4) disparities in healthcare (e.g., many healthcare services are inaccessible, inadequate, or discriminatory) leading to compromised health and mental health outcomes; and (5) legal capacity (e.g., reduced opportunities for self-determination in decision-making robs them of autonomy and directly infringes on their right to make critical decisions that affect their life).

The United Nations Convention on the Rights of Persons with Disabilities (CRPD; United Nations, 2006) has been ratified by many countries to safeguard the rights of people with IDD. Among the Articles of the CRPD (see Table 1), these five are probably most pertinent for safeguarding the rights of people with IDD in terms of treatment and care provisions:

- (1) *Article 12: Equal recognition before the law.* The CRPD mandates that people with disabilities, which includes those with IDD, should have the legal capacity equal to those of people without disabilities. Article 12 asserts that irrespective of their level of cognitive functioning, people with IDD have the right not only to make legal, health, lifestyle, and other decisions but also to have those decisions respected. This requires a shift from the traditional guardianship and conservator models to supported participatory decision-making (Shogren & Singh, 2022).
- (2) *Article 15: Freedom from torture and cruel, inhuman, or degrading treatment or punishment.* This Article forbids the use of treatments, interventions, and practices that can be construed as inhumane or degrading for people with IDD (see Nisbet & Weiss, 2021). This would include seclusion, restraints, aversive practices (e.g., electric shock, punishment, and certain psychotropic medications which have historically been used as chemical restraints).
- (3) *Article 19: Living independently and being included in the community.* People with IDD have the right to live and participate in the community, with choices that are available to everyone. They are entitled to choose their residence, with whom they wish to live, and the nature of their living arrangements. This requires access to a range of in-home and community support services that are responsive to their needs and would preempt social isolation or segregation in the community.

Table 1 Articles of the Convention on the Rights of Persons with Disabilities (CRPD)

Preamble
Article 1 – Purpose
Article 2 – Definitions
Article 3 – General principles
Article 4 – General obligations
Article 5 – Equality and non-discrimination
Article 6 – Women with disabilities
Article 7 – Children with disabilities
Article 8 – Awareness-raising
Article 9 – Accessibility
Article 10 – Right to life
Article 11 – Situations of risk and humanitarian emergencies
Article 12 – Equal recognition before the law
Article 13 – Access to justice
Article 14 – Liberty and security of person
Article 15 – Freedom of torture or cruel, inhuman or degrading treatment or punishment
Article 16 – Freedom from exploitation, violence and abuse
Article 17 – Protecting the integrity of the person
Article 18 – Liberty of movement and nationality
Article 19 – Living independently and being included in the community
Article 20 – Personal mobility
Article 21 – Freedom of expression and opinion, and access to information
Article 22 – Respect for privacy
Article 23 – Respect for home and the family
Article 24 – Education
Article 25 – Health
Article 26 – Habilitation and rehabilitation
Article 27 – Work and employment
Article 28 – Adequate standard of living and social protection
Article 29 – Participation in political and public life
Article 30 – Participation in cultural life, recreation, leisure and sport
Article 31 – Statistics and data collection
Article 32 – International cooperation
Article 33 – National implementation and monitoring
Article 34 – Committee on the Rights of Persons with Disabilities
Article 35 – Reports by States Parties
Article 36 – Consideration of reports
Article 37 – Cooperation between States Parties and the Committee
Article 38 – Relationship of the Committee with other bodies
Article 39 – Report of the Committee
Article 40 – Conference of States Parties
Article 41 – Depositary
Article 42 – Signature
Article 43 – Consent to be bound
Article 44 – Regional integration organizations
Article 45 – Entry into force
Article 46 – Reservations
Article 47 – Amendments
Article 48 – Denunciation
Article 49 – Accessible format
Article 50 – Authentic texts

Note: Of the 50 Articles, #5 to #30 are seen as being critical for enhancing equality, dignity, autonomy, empowerment, and quality of life for people with disabilities. The other articles are administrative in nature

(4) *Article 24: Education.* This Article addresses the right to education, which should be anchored in an inclusive education system that addresses the diverse needs of people with IDD. It assumes the education of all students

in inclusive schools and eschews segregated education of students with IDD. This requires resource-intensive teaching methods and commensurate curriculum.

- (5) *Article 25: Health. People with IDD should have the right to the highest attainable standard of health.* By implication, this Article focuses on the disparities in healthcare access and treatment that are often endured by those with IDD. This requires community health providers to be knowledgeable about ways to elicit treatment preferences of people with IDD and to co-determine what treatment and intervention methods would work best for each person. It would require a change in attitude of the health care providers in terms of acquiring skills needed to provide the necessary treatment in a manner commensurate with the person's expectations.

The CRPD provides a blueprint for nations to significantly improve the lives of people with IDD. It recognizes and reinforces their inherent dignity, autonomy, and full citizenship rights. It strongly advocates their legal rights, freedom from harmful treatments and interventions, their right to live and fully participate in community activities, inclusive education, and to receive the same level of health care as everyone else in the community. Furthermore, it requires all nations to provide the necessary supports and services to translate the intent of the CRPD Articles into reality so that people with IDD can live with dignity and enjoy meaningful lives. The CRPD has been instrumental in producing many positive changes in the lives of some people with IDD. However, a systemwide transformation is yet to take place because it presents a complex and costly challenge to change the mindsets of politicians, administrators, service providers, and families to translate the philosophy and principles of the CRPD into living reality for all people with IDD.

Disability

Disability is a broad term used to describe impairments that people live with on a daily or intermittent basis. It has been defined in various ways, with the Centers for Disease Control and Prevention (2020) defining it as “. . . any condition of the body or mind (impairment) that makes it more difficult for the person with the condition to do certain activities (activity limitation) and interact with the world around them (participation restrictions).” According to the World Health Organization (2001), disability has three dimensions: (1) Impairment in body structure or function, or mental functioning (e.g., scoliosis, loss of hearing, or memory); (2) Activity limitation (e.g., difficulty in walking, hearing, or seeing); and (3) Participation restrictions in normal daily activities (e.g., engaging in social and recreational activities). The CRPD emphasizes that “. . . disability is an evolving concept and that disability results from the interaction between persons with impairments and attitudinal and

environmental barriers that hinder their full and effective participation in society on an equal basis with others.”

Disabilities have been classified in diverse ways based on agency criteria for understanding the nature of the disabilities and the development of necessary supports and services. Frequently, disabilities are conceptually classified based on specific defining impairments. For example, they can be categorized as: (1) Physical disabilities, encompassing impairments affecting mobility and bodily functions, such as cerebral palsy, loss of a body part, and paralysis; (2) Sensory disabilities, covering impairments in any of the five senses, such as deafness or hard of hearing and blindness or low vision; (3) Cognitive disabilities, involving impairments in thinking, learning, and processing information observed in autistic individuals, people with IDD, and those with specific learning disabilities; (4) Mental health disabilities, incorporating conditions listed in the Diagnostic and statistical manual of mental disorders (DSM-5-TR; American Psychiatric Association, 2022); (5) Acquired disabilities, resulting from injury, illness, or environmental factors, including traumatic brain injury and spinal cord injury, among others; (6) Invisible disabilities, stemming from hidden conditions leading to chronic pain, chronic fatigue syndrome, and some mental health conditions; and (7) Developmental disabilities, diagnosed before the age of 18 years, encompassing lifelong conditions such as Down syndrome, fetal alcohol syndrome, and many others, with people with IDD falling into this classification.

The understanding of disability in the context of people with IDD has undergone significant evolution over time (Donoghue, 2003; Fitzgerald, 2006; Lo-Bianco & Shepard-Jones, 2008), beginning with religious discourses and progressing to scientific conceptions (Humpage, 2007). Various models of care have been conceptualized and implemented, often by professionals and advocates dedicated to the wellbeing of people with IDD (Brittain, 2004). Only recently people with IDD have begun actively advocating for a reevaluation of how society perceives disabled people and for the nature of services they require as full citizens. It is informative to briefly explore some of models of services to understand how they have shaped societal perceptions of people with IDD and how these perceptions have been, and continue to be, translated into interventions and services for them. In addition, the way disabilities are perceived influences people's expectations and interactions with people with IDD (Barton, 2009), making it a crucial aspect of any discussion on human rights.

Medical Model. In this model, disability is perceived as a medical condition or ailment, placing medical professionals as the specialist experts responsible for treating and alleviating both the symptoms and the underlying diseases or disorders affecting people with IDD (McTigue, 2015).

People are categorized as disabled if their impairment hinders them from performing tasks considered “normal” in comparison to those without disabilities (Blustein, 2012). Viewing disability from a biological standpoint, the medical model sees it as a biological issue independent of external factors such as the environment, societal values, culture, or politics. Disability, according to this model, results from limited functioning due to bodily impairments, encompassing issues related to body function, structure, and cognitive limitations stemming from physical, sensory, emotional, or cognitive challenges (Palmer & Harley, 2012). The medical model gave rise to the assumption that people with IDD could overcome or at least mitigate their disabilities only by engaging in the treatments and services provided by medical doctors and other healthcare professionals.

One major criticism of the medical model is that medical experts extended their influence beyond treating disabilities to having control or authority over the lives of people with IDD, shaping perceptions of how society values such people (Haegele & Hodge, 2016). Diagnoses and labels provided by medical personnel led to determinations of the services offered, their types, how they were provided, and the benefits of such services, often overlooking the values, desires, and aspirations of people with IDD (Barton, 2009; Humpage, 2007). Another significant critique is that by associating disabled people with a sick role and framing disability in terms of deficits (Mitra, 2006), the medical model perpetuated stereotypes portraying them as abnormal and feeble. Consequently, this portrayal has influenced negative perceptions, and condescending attitudes and behaviors of society towards people with IDD (Goering, 2015). These considerations led to the development of social models of disability.

Social Model. The social model of disability, introduced by Oliver (1983) as an alternative to the prevailing medical model, has since seen various iterations (Mitra, 2006; Pfeiffer, 2001). The overarching social model posits that societal attitudes, discrimination, and exclusion are the primary contributors to the marginalization of disabled people, including those with IDD (Winance, 2016). According to this model, disabled people do not face barriers in life due to their mental and physical impairments; rather, these challenges arise from environmental barriers imposed by societal, cultural, political, and attitudinal beliefs (DePoy & Gilson, 2015). In essence, disability can be significantly reduced or even eliminated if society provides the necessary supports and services (Bailey et al., 2015). In contrast to the medical model, the social model situates disability within the socioeconomic environment rather than within the individual, recognizing impairments as a natural part of human diversity. Discrimination against disabled people is considered a violation of their human rights (Coles, 2001; Palmer

& Harley, 2012). The social model implies that the limiting factor is not the impairments of the individual but society itself, which imposes disability on individuals with IDD by not removing environmental barriers they face and by holding ill-informed perceptions that they are incapable of full participation in society (LoBianco & Sheppard-Jones, 2008). The practical implication of these views is that the lives of people with IDD can be greatly enhanced through political, economic, and social transformations of society itself.

A critical element of the social model is its complete separation of impairments from disability, a point that critics argue is a significant flaw. They contend that the experiences of disabled people, influenced by their impairments, significantly affect their interaction with various aspects of society (Palmer & Harvey, 2012). Individual impairment is an observable attribute of a disabled person and is intricately linked to their lived experience. Another critique centers on the social model's failure to account for variations among disabled people, specifically overlooking the intersectionality of different oppressive forces such as race, ethnicity, gender, sexual orientation, and so on (Fitzgerald, 2006; Reindal, 2008, 2009). Advocating for full societal accommodation of disabled people, without considering these intersecting factors, may not adequately capture the lived experience of disabled individuals who cannot escape discrimination related to these issues. From a social creationist perspective, societal prejudices may not merely be the collective beliefs of society, but a more insidious form of institutionalized discrimination (Oliver & Barnes, 2012). These and related criticisms of the social model have led to calls for re-examination and re-invigoration of the scope and applications of the model (Levitt, 2017; Riddle, 2020; Woods, 2017).

Biopsychosocial Model. This model, developed by Engel (1977), aimed to bridge the gap between the psychoanalytic and biological conceptualizations of mental disorders in psychiatry. Grounded in science, this model extended beyond the scientific realm to combat the dehumanization and disempowerment of individuals with mental disorders through empathy and compassion in medical practice (Engel, 1980, 1988). This holistic approach to disability recognizes the intricate and dynamic interplay of biological, psychological, and social factors that determine human functioning (Borrell-Carrió et al., 2004). Biological factors may encompass physical health, such as genetic predispositions, medical conditions, physical conditions. Aging, including age-related changes in the body, and gender, with biological differences impacting susceptibility to specific disabilities, are also considered. Psychological factors include mental health considerations affecting cognitive and emotional wellbeing, such as depression and anxiety.

Coping strategies, resilience, and attitude towards one's disability, along with self-perception affecting self-esteem and self-efficacy, are also part of the psychological dimension. Social factors encompass societal discrimination, reflecting social attitudes and prejudices, as well as accessibility factors affecting functioning (e.g., accessible public spaces, transportation, assistive and prosthetic devices), and social support (e.g., from family, friends, and community). These three global factors are interrelated and interact dynamically, with changes or disruptions in one factor influencing the others.

The biopsychosocial model provides a comprehensive understanding of people with disabilities and offers a methodology for clinical intervention. While considered a strength by some, critics argue that its comprehensiveness makes it challenging in applying the model to community care for disabled people. Despite the existence of a biopsychosocial formulation manual (Campbell & Rohrbaugh, 2015), significant variations in its use across disabilities have been observed. This variability is expected given the wide range of disabilities evidenced in our community, but the disparities in its application across agencies for the same disability are somewhat disconcerting. Another criticism is that, in certain healthcare and rehabilitation settings, the medical aspect of the model tends to overshadow the psychological and social components due to administration by medical personnel. At the same time, others argue that the psychological and social aspects of disabilities are often overemphasized without due consideration of biological factors. The precedence given to each component depends on the clinical strengths of professionals, highlighting the necessity of a strong treatment team leader to provide an integrative and nuanced understanding of each person's disability. Moreover, some critics note a potential inadequacy in the model's emphasis on structural factors in the community that impose difficulties for disabled people. These factors discriminatory policies, inaccessible infrastructure, social stigma, and limited resources and services for specific disabilities.

Human Rights Model. In contrast to the other disability models developed by advocates or clinicians, the Human Rights Model emerges from the concepts of equality and discrimination as articulated by the CRPD (Degener, 2016a, b, 2017a, b; Lawson & Beckett, 2021). As highlighted by Degener (2016b), the CRPD essentially encapsulates the human rights model of disability. The initial articulation of the human rights model emphasizes that "Human dignity is the anchor norm of human rights. Each individual is deemed to be of inestimable value, and nobody is insignificant. People are to be valued not just because they are economically or otherwise useful but because of their inherent self-worth. . The human rights model focuses on the inherent dignity of

the human being and subsequently, but only, if necessary, on the person's medical characteristics. It places the individual center stage in all decisions affecting him/her and, most importantly, locates the main "problem" outside the person and in society" (Quinn & Degener, 2002, p. 14). In a broader context, "the end goal from the perspective of the human rights model is to build societies that are genuinely inclusive, societies that value difference and respect the dignity and equality of all human beings regardless of difference" (Quinn & Degener, 2002, p. 15).

The details of the human rights model are expounded by Degener (2016a) and the following six propositions differentiate it from the medical, social, and biopsychosocial models (Degener, 2016b, pp. 3–14):

- (1) Disability is a social construct, but human rights do not necessitate a specific health or body status (i.e., the absence of impairment is not a prerequisite for human rights because impairment does not impede a person's human rights capacity).
- (2) Human rights encompass more than anti-discrimination (i.e., they include civil and political rights, as well as economic, social, and cultural rights).
- (3) Impairment is recognized as human variation (i.e., disabled people must confront the spectrum of pain and suffering, similar to everyone in society).
- (4) Multiple discrimination and layers of identity must be acknowledged (i.e., identity politics should be a component of disability policy, considering minority and cultural identification, including race, ethnicity, gender, sexual orientation, and the various impairment categories).
- (5) Prevention policy should be sensitive to human rights (i.e., prevention policy is seen as human rights protection for disabled persons).
- (6) Poverty and disability are interrelated, but there is a roadmap for change (i.e., disability is a development issue covered in Article 11 of the CRPD, providing a pathway for change in the daily lives of disabled persons as well as during natural disasters and humanitarian emergencies).

The human rights model corresponds to explicit articles of CRPD. For instance, it emphasizes the importance of involving disabled persons in decision-making processes that affect their lives (Article 4) and advocates for the elimination of barriers in all areas of life, such as education, employment, transportation, and public services to ensure full participation and accessibility for disabled individuals (Article 9). Moreover, the human rights model of disability is already being used in community and research settings

to make a difference in the lives of disabled people (e.g., Lombard-Vance, 2023).

In sum, disability is a dynamic concept continually evolving and shaping societal attitudes, influencing policies, and impacting practices for all disabled people, including those with IDD. As knowledge and attitudes toward disability undergo transformations, existing models have been revised or new ones developed to incorporate new insights derived from philosophy, service practices, and outcome data reflecting changes in the quality of life for disabled people. In this context, the emergence of new models parallels the development of new theories in science (Kuhn, 1996). Undoubtedly, the medical, social, biopsychosocial, and human rights models are not the last models of disability; newer models and approaches are already emerging. For example, the social-ecological model (McLeroy et al., 1988) has recently been adapted within a human rights context to account for the adaptive behavior of people with IDD when responding to maladaptive environments (Jorgensen et al., 2023).

Mindfulness

Mindfulness, in everyday parlance, means doing something consciously, carefully, or attentively. In terms of Buddhist teachings, mindfulness is a somewhat limited translation of the Pāli word *sati*, or its Sanskrit equivalent *smṛiti*. It has been variously defined by Buddha in his teachings and commentaries by contemporary teachers and academics (see Amaro & Singh, 2021). The most cited definition of mindfulness is by Kabat-Zinn (2003, p. 145): “the awareness that emerges through paying attention on purpose, in the present moment, and nonjudgmentally to the unfolding of experience moment-by-moment”. Although there are hundreds of different forms of meditation practice (Matko et al., 2021), the two key mindfulness meditation practices are *Samatha* (tranquility) and *Vipassanā* (insight). As expressed in the *Aṅguttara Nikāya* [2:13]:

Two things, O monks, partake of supreme knowledge. What two? Tranquility and insight.

If tranquility is developed, what benefit does it bring? The mind becomes developed. And what is the benefit of a developed mind? All lust [desire] is abandoned.

If insight is developed, what benefit does it bring? Wisdom becomes developed. And what is the benefit of developed wisdom? All ignorance is abandoned.

A mind defiled by lust is not freed; and wisdom defiled by ignorance cannot develop. Thus, monks, through the fading away of lust there is liberation of mind; and

through the fading away of ignorance there is liberation by wisdom.

Samatha meditation leads to calmness or serenity, often interpreted as resting the mind. *Vipassanā* meditation facilitates insight into the true nature of reality, focusing on impermanence, dukkha, and not-self. That is, *Vipassanā* focuses on the nature of the mind.

Mindfulness and mindfulness-based interventions have been extensively researched for their effectiveness in treating conditions arising from human disabilities, including physical, sensory, psychological, and cognitive disabilities (see Singh et al., 2024 for a conceptual review). Individuals with cognitive disabilities, such as those with IDD, often face above-average difficulty with mental tasks and may also contend with co-occurring physical and mental health issues. People with IDD encounter multidimensional challenges in everyday life, which can be attenuated with appropriate supports and services. One of these services includes mindfulness-based interventions, which have been shown to enhance their quality of life.

Mindfulness-based intervention can be utilized in two general ways with people with IDD. First, they can serve as adjuncts to an array of evidence-based therapeutic and educational interventions, including:

- (1) Educational interventions (e.g., individualized education plans in special and regular education).
- (2) Behavioral interventions (e.g., applied behavior analysis and positive behavior support).
- (3) Speech and language therapy for those with communication difficulties.
- (4) Occupational therapy for those who have difficulties with daily living skills,
- (5) Cognitive rehabilitation interventions (e.g., cognitive training in functional skills and compensatory skills training),
- (6) Assistive technologies (e.g., augmentative and alternative communication devices, text-to-speech, and speech-to-text software), and.
- (7) Vocational and life skills training (Singh et al., 2024).

Second, mindfulness-based programs can be employed as the primary intervention, either with their caregivers as mediators of behavior change or directly with those with IDD (Felver et al., 2022; Hwang & Singh, 2023; Singh & Hwang, 2020, 2021; Singh & Singh Joy, 2021; Singh et al., 2022).

The alignment of mindfulness-based interventions with specific Articles of the CRPD is questionable because extant research has not been firmly grounded within a human rights model. This does not necessarily imply that certain

tenets of disability models (e.g., medical, social, biopsychosocial, and human rights models) have not been addressed. Indeed, medical interventions and related services for people with IDD often encompass key aspects of the medical model. Furthermore, the services mandated by social and biopsychosocial models have shaped the provision of many current community-based services. What is suggested here is that researchers engaged in mindfulness-based interventions may either be unaware of the CRPD or have not specifically anchored their interventions and services in specific Articles of CRPD. Given the recent applications of mindfulness in the field of IDD, current research has predominantly focused on analyzing the correlates, moderators, predictors, and outcomes of mindfulness-based interventions.

One of the key issues is to raise awareness within the mindfulness research community regarding the essential Articles of CRPD and the necessity for further research in these areas. Following this awareness, efforts should be directed towards developing a consensus on the key elements of each relevant CRPD Article, which can be operationalized and defined. This, in turn, will facilitate the establishment of standards and measurement criteria for assessing the degree of alignment of past, present, and future research with the CRPD Articles. Similar to existing criteria for assessing research quality (e.g., Cochrane intervention reviews, Higgins et al., 2023), the establishment of quality standards for alignment with the Articles of CRPD is crucial for advancing research and practice in mindfulness.

Fully integrating the aims of the CRPD and mindfulness-based interventions for people with IDD will require overcoming intellectual and practical hurdles. An example of such challenges is determining how to develop and implement mindfulness practices for people with IDD who face the most significant impairments, particularly those at the severe and profound levels of functioning. Adhering to the principle *Nihil de nobis, sine nobis* (“Nothing about us without us”) aiming for full participation and equal opportunity for, by, and with people with IDD who live with the highest levels of cognitive and physical disabilities, necessitates finding ways to provide care and services without being paternalistic and overprotective. Addressing the wants and needs of individuals with disabilities, especially those with severe impairments, to achieve a good quality of life may significantly differ from recommendations by professionals and what is offered by service providers. While this perspective might not yet be prominent in the awareness of mindfulness researchers, it could be beneficial to adopt a disability perspective and the human rights model of disability as part of the theoretical framework for obtaining informed consent, provision of mindfulness-based services, and in participatory research in mindfulness.

Data Availability This is a review article, and no data are included.

Declarations

Ethics approval This is a review paper and does not require an ethics approval.

Conflict of interest The authors declare no conflict of interest.

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